CyncHealth

June 16, 2025

Centers for Medicare & Medicaid Services
P.O. Box 8013
Baltimore, MD 21244-8013

RE: CMS-0042-NC. RFI-Health Technology Ecosystem

Dear Assistant Secretary,

CyncHealth appreciates the opportunity to provide our comments on the Centers for Medicare & Medicaid Services (CMS), Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC), Department of Health and Human Services (HHS) recent Request for Information on the Health Technology Ecosystem. We commend the agencies for engaging stakeholders in shaping the future of the national health technology ecosystem.

As a leading health data utility (HDU) and the designated health information exchange (HIE) for Nebraska and Iowa, CyncHealth is dedicated to advancing interoperability, improving healthcare outcomes, and empowering both patients and providers through secure and comprehensive data exchange. Our network connects more than six million patient lives and over 1,100 healthcare facilities including hospitals, clinics, long-term care centers, and public health agencies across the Midwest. This extensive reach enables the seamless flow of critical health information, supporting informed decision-making and better care delivery.

We have carefully reviewed the questions posed in the RFI and have focused our responses on those most relevant to HIEs. CyncHealth strongly supports the effective and responsible adoption of health technology that empowers patients to make better decisions for their health and well-being, while also enhancing provider workflows and supporting community health initiatives

Providers frequently report that document-based exchange results in information overload and inefficiency, as the process of searching for specific results or discrete data within lengthy documents is time-consuming and often impractical at the point of care. To establish a resilient and effective health technology ecosystem, it is essential to prioritize true interoperability, long-term sustainability, and meaningful patient empowerment.

Our recommendations to CMS/ASTP/ONC emphasize:

- **Leverage Proven HIE/HDU Models:** Document exchange networks and claims data are insufficient for improving cost and quality. ASTP should build upon established HIE/HDU frameworks that have demonstrated success in facilitating actionable real time data exchange.
- **Enforce Consistent Standards**: Ensure the real world adoption and enforcement of uniform data standards to enable the exchange of health information.

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- **Promote Innovation through Open Application Programming Interfaces APIs:** Support the development and use of open APIs to foster innovation, improve usability, and enhance integration across diverse systems.
- **Ensure Equitable Funding:** Provide financial support to sustain and expand HIE/HDU infrastructure, particularly in underserved and rural areas.
- **Align Incentives and Reduce Barriers:** Create guidelines that align incentives for all stakeholders to encourage broad participation.

We look forward to continued collaboration with CMS, ASTP/ONC, and other stakeholders to advance a more connected, cost-effective, and patient-centered health technology ecosystem.

Sincerely,

Jaime Bland, DNP, RN
President and Chief Executive Officer of CyncHealth

402-506-9900 CyncHealth.org PO Box 27842 | Omaha, NE 68127

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CyncHealth Overview and Organizational Perspective

CyncHealth is structured as a public-private partnership and leading Health Data Utility providing data and interoperability solutions for people, providers, and public health in Nebraska and Iowa. In addition to operating the HIEs in both states, we offer the prescription drug monitoring program (PDMP) in Nebraska and offer a suite of interoperable public health and social care data-sharing services in both Nebraska and Iowa. At CyncHealth, we believe connected care is better care; we seek to create a better experience for patients and providers while easing burdens and reducing costs.

Response to Key RFI Questions

Patients & Caregivers

Reducing Administrative Burden and Advancing Value-Based Care (PC-5b)

To enable timely access to high-quality CMS and provider-generated patient data, two key technological improvements are essential. Implementation of federated identity and single sign-on solutions would significantly reduce access friction for healthcare providers while supporting higher utilization rates of HIE services, creating a more streamlined approach to patient data retrieval. CyncHealth has demonstrated the success of this capability, easing clinicians' burdens by providing a unified SSO experience for accessing the Health Data Utility in both urban and rural communities. By onboarding Nebraskan participants in high-volume health networks to a federated SSO experience, CyncHealth has further reduced the clicks needed to access critical patient care data. This commitment extends to all participants, with a federated SSO project underway for the Iowa Health Data Utility to enhance efficiency regardless of the underlying technology.

Building on this foundation, CyncHealth's interoperability infrastructure demonstrates proven scalability through the Iowa Bed Dashboard, which processes real-time hospital ADT data statewide with message volumes increasing 3000% since 2021. This adaptable platform now supports multiple applications including CMS compliance reporting, care coordination, and syndromic surveillance, establishing a reusable framework that reduces administrative overhead while advancing value-based care initiatives and creating clear pathways for expanded federal partnerships.

Additionally, better workflow integration capabilities would reduce the need for redundant data entry and manual record retrieval processes, ultimately enabling more efficient care delivery that supports value-based, patient-centered care models. These changes would help create a smoother data system, allowing providers to quickly access complete patient information, leading to better care and more efficient operations.

Data Quality, Standards, and Equity (PC-8, PC-9)

In our experience, addressing data quality, standards, and equity challenges reveals a persistent issue; health data are abundant, yet significant gaps in standardization remain, particularly among smaller healthcare facilities that struggle to implement data standards due to cost constraints and vendor limitations. Even large EHR vendors often delay or resist adopting new standards, creating inconsistencies that hinder data exchange. Vendor accountability remains a critical issue, as EHR vendors should be held accountable for meeting data quality and interoperability standards, especially when charging additional fees for data connections.

To achieve widespread improvement, states should be required to designate and support statewide health information exchanges (HIE) or health data utilities (HDU), with statutory mandates for provider participation to ensure comprehensive data coverage and reduce friction among stakeholders. This coordinated approach would create the infrastructure necessary for meaningful data sharing while maintaining appropriate privacy protections and ensuring equitable access to health information across all communities.

The integration of comprehensive care data requires including social determinants of health and other non-clinical information that significantly impact patient outcomes. Best practices exist and should be highlighted for similar success, such as CyncHealth's demonstrated efforts in data quality and completeness when proper standards and incentives align. CyncHealth is currently working with 44 sites that are either live or in progress, demonstrating the possibility of facility and HDU partnership in improving data quality and completeness.

CyncHealth's model also shows how research can be fundamentally integrated into community health support. CyncHealth supports community research, including studies on behavioral health intervention outcomes, maternal-child health, care continuity and health outcomes, and medication safety through our comprehensive, longitudinal health record which includes medication and clinical data. Our output regularly aligns with publicly available data, while ensuring the data is actionable using near-real time output. Further, our data allows researchers, policy analysts and community organizations to ensure they have the necessary support to understand the health IT infrastructure and health data complexities, without data silos or disparities across systems. We believe this model is scalable to answer larger questions and support health care delivery more broadly.

Performance Metrics and Accountability (PC-10b/e/f/g)

To enhance TEFCA, several key improvements are needed to leverage the strengths of existing HIE infrastructure. First, National HIE Performance Guidelines should be established, as there is currently a lack of federal standards for evaluating HIE performance. These guidelines would support accountability, transparency, and continuous improvement across the network. Second, Reimbursement Clarity is essential as clear reimbursement guidelines for HIE services ensure financial viability and encourage meaningful provider engagement in both TEFCA and complementary HIE networks. Finally, Data Completeness and Quality standards must be addressed through a national framework that includes minimum data quality standards and

reporting requirements to ensure consistency across HIEs operating within and alongside TEFCA.

A critical concern is that document exchange at the point of care does not provide a usable, cohesive health record but rather serves primarily to fill care gaps for payers. This approach creates a significant burden on providers who must parse, normalize, and sort the data, making it difficult to use effectively in clinical workflows.

CyncHealth believes that established HIEs and eHealth Exchange networks provide essential complementary infrastructure to TEFCA's national framework. Strategic investments should focus on strengthening these proven systems, working seamlessly with TEFCA, creating a comprehensive interoperability ecosystem that requires fewer redundant connections and places less burden on points of care. Moreover, TEFCA implementation should incorporate the valuable experience HIEs have gained in handling state laws and regulations in their daily operations. Pursuing vendor-mediated solutions through TEFCA without leveraging existing HIE expertise may overlook states' rights and unnecessarily complicate patients' ability to opt out of national networks, potentially undermining patient autonomy and state-level governance that HIEs have successfully managed.

Patient Access and Empowerment (PC-11/12)

HIEs are currently advancing patient access to health information through several initiatives that address persistent barriers in healthcare data sharing. By providing comprehensive longitudinal health records that follow patients across different providers, HIEs ensure continuity of care. They also use terminology services to standardize data from diverse sources, making medical information more understandable for patients. CyncHealth has successfully implemented a longitudinal health record across Nebraska and Iowa, establishing connections with all hospitals, pharmacies and most clinics that use EHRs. Providers are now routinely accessing this information as part of their regular workflows. These efforts were recognized in 2025 when CyncHealth was named on the Sequoia Project Interop100 list. While document-based networks have fallen short by merely indicating the presence of data, rather than offering seamless access to usable health records, HIEs bring together fragmented data silos and the value of accessible patient information to support coordinated care delivery.

HIEs also offer patients independence from any single EHR vendor by providing access to their complete health data regardless of which EHR system their various providers use, significantly enhancing data portability and transparency. At CyncHealth we believe in the implementation of unified, provider-agnostic patient portals that allow patients to access all their health records across different providers through a single interface, including the ability to manage records for dependents. We also believe the most valuable operational health data use cases for patients and caregivers are in the value of care coordination, addressing care gaps and establishing unified patient records for network usability. These capabilities enable patients and caregivers to navigate care more efficiently by eliminating redundant testing and ensuring seamless transitions between specialists, primary care, and health systems.

While we look toward the near future, the United States needs a fully interoperable network that can support not only routine care coordination but also rapid response capabilities essential for pandemic preparedness and public health emergencies needs that can only be met by accessible, discrete, structured data elements.

Information Blocking and Digital Identity (PC-14)

When establishing digital identity credentials and the roles that CMS/payers, providers, and app developers have in driving adoption, the key priority is ensuring that identity credentials are portable across different sectors and applications. For example, digital credentials used for boarding a plane should be accessible by healthcare providers if the patient chooses to share them, creating a seamless and interoperable identity verification system.

CyncHealth has found success in patient matching using the Enterprise Master Patient Index (EMPI) system. The EMPI system utilizes data from external sources for individual patient identifiers and demographics in the matching process. The matching engine calculates a weight between individual source records and enterprise curated records. If the calculated weight is above a high threshold, the added or updated records are automatically linked. Various false positive rules are used to prevent automatically linking records that are NOT related.

If the match weight falls between the high and low thresholds, the comparison is classified as a potential match, requiring manual investigation and resolution. The matching engine also uses external referential data to determine if potential matches should be linked. Field agreement and disagreement weights are calculated for name, date of birth, and various demographic fields. Weights for all field comparisons are summed to form a composite weight.

CyncHealth's approach ensures that identity credentials are portable across different sectors and applications, creating a seamless and interoperable identity verification system. This success in patient matching has been a key priority for CyncHealth, ensuring that digital credentials used for boarding a plane can be accessible by healthcare providers if the patient chooses to share them.

Providers

Innovation and App Ecosystem (PR-1/2)

Healthcare providers face significant responsibilities and encounter substantial obstacles when it comes to digital health applications and innovation. When recommending digital products to patients, providers must inform patients if the app sells data in its terms of use, ensuring transparency about data monetization practices that could affect patient privacy.

However, barriers to app development present major obstacles that prevent the development and implementation of the most useful and innovative applications for physician workflows, including quality measurement reporting and clinical documentation. EHR vendors often act as gatekeepers, creating costly and slow approval processes that hinder innovation and restrict the development of tools that could enhance clinical workflows and patient care.

To overcome these obstacles, ONC should promote open, fair marketplaces for third-party apps, similar to mobile app ecosystems, to foster innovation and improve clinical tools. Additionally, bidirectional API requirements are essential as current certification requirements mandate data exposure via FHIR; they do not require data ingestion capabilities. ONC should mandate certified systems that are both capable of sending and receiving data using standardized APIs and ensure that data is usable through proper parsing and deduplication processes. These changes would create a more competitive and innovative environment that benefits both providers and patients through improved clinical tools and workflows.

Data Exchange (PR-5/7/8)

Patient access APIs are essential because they empower individuals to view and manage their own health information. It is critical that access to this data be treated as a public service to protect patients from having their information monetized or reused without their control. Ideally, this access should be available through HIEs, which can also manage a centralized consent registry to eliminate the burden of repeated opt-ins or opt-outs. All downstream systems, including vendor networks, must be required to honor these patient consent preferences.

A strategy CMS can implement to support providers in delivering high-quality, timely, and comprehensive healthcare data for interoperability in the digital health ecosystem is to update and strengthen existing standards. Current standards are not achieving their intended goals. Consolidated Clinical Document (CCD) outputs often lack critical data, and HL7 feeds remain heavily dependent on clinical workflows, leading to inconsistencies. When data is incomplete, the issue is attributed to data quality across the network. Although EHR systems may be certified for interoperability, they are often unable to perform effectively in real-world scenarios. Updating standards to reflect real-world implementation challenges is essential to achieving interoperability.

HIEs offer valuable services to support providers, but there is a need for CMS to clearly define what success looks like for HIEs and networks, particularly in the context of simplifying providers' responsibilities around clinical quality data. Currently, many data exchange networks are designed with a focus on technology rather than the needs of providers, point-of-care decision-making, or the data requirements of alternative payment models. This misalignment often makes it difficult for providers to access the data they need when they need it.

To address this, CMS should consider tasking HIEs with the development and maintenance of longitudinal health records that fulfill the requirements for clinical quality reporting, patient access, public health monitoring and other outcome-based initiatives. While registries may not directly impact clinical care they significantly contribute to public and population health. Many HIEs already perform these functions as part of their core operations, positioning them well to lead in the creation of a more efficient data ecosystem. Achieving this vision will require a comprehensive, well-aligned national data strategy.

Digital Identity (PR-9)

To encourage providers to accept digital identity credentials from patients and their partners instead of requiring proprietary logins for each provider relationship, CMS needs to be involved in the development of a coherent, comprehensive data strategy as part of a broader HHS initiative. Currently, providers face excessive administrative burdens from multiple agency data requests, often requiring the same information to serve different purposes within various agencies. By establishing standardized digital identity credential acceptance across CMS programs and reducing duplicative data collection requirements, the agency could create significant efficiencies in both data requests and management processes. This approach would eliminate the need for patients to maintain separate login credentials with each provider while streamlining provider workflows and reducing administrative overhead. However, any strategy must respect state rights, ensuring that federal efforts support rather than replace state control over healthcare data and identity system.

Payers (PA-5b)

To consolidate and manage numerous use cases and data requests for providers serving both CMS and other payers, there must be a structured approach to creating aggregated records at the state HIE level with streamlined processes for data requests across agencies. Rather than managing numerous duplicative data requests that exceed CMS requirements, states should build unified infrastructure that serves multiple purposes - for example, using ADT feeds for both Syndromic Surveillance and lab feeds for electronic lab reporting (ELR).

This approach reduces redundancy in requests and regulations while building robust state-level infrastructure. Investments can be dual-purpose and meet various payer requirements while significantly reducing the administrative burden on providers. The federal focus should remain on establishing structure and outcomes while allowing states flexibility to implement solutions that work effectively within their unique healthcare landscapes.

Technology Vendors, Data Providers, & Networks

Ecosystem (TD-2c)

One of the main obstacles to accessing healthcare data today is the complexity of aggregating information across multiple systems. While APIs have enabled access to be more feasible, they often still require manual data collection from various portals. For example, providers may need multiple logins to systems like Epic, Cerner, and Meditech, which falls short of achieving true interoperability. This fragmented approach increases provider burden but also drives up costs. To address this, a state-based infrastructure that allows individuals to control how their data is used could streamline access and simplify the overall system.

TEFCA and Interoperability Frameworks (TD-4/6/10)

CMS can strengthen interoperability by continuing to promote the use of open, standards-based, publicly available APIs instead of proprietary ones. Rather than creating new systems from scratch, CMS should focus on building upon existing infrastructure and increase the use of HIEs

and HDUs as they already support interoperability at the state, regional, and national levels. Reinventing the wheel risks creating further fragmentation and inefficiencies.

CMS should also consider supporting a public-private partnership model as a compliment to TEFCA. A national framework of non-profit, public-private partnerships, such as CyncHealth, would demonstrate success when guided by consistent federal guidelines. These models promote long-term sustainability, local accountability, and responsiveness to community health needs. The existing alternative includes state-based Enterprise Master Patient Index (EMPI) systems managed through HIEs, which can serve as a foundation for advancing nationwide interoperability goals.

Data Exchange (TD-17)

Given operational costs associated with healthcare data sharing networks, CMS should provide clear, comprehensive guidance on data sharing requirements across all HHS agencies to eliminate confusion and duplicative efforts. CMS should also establish grant requirements that mandate data sharing, creating financial incentives to encourage participation while ensuring sustainable network operations.

Value Based Care Organizations

Digital Health Adoption (VB-3)

Essential health IT capabilities for value-based care arrangements include the ability to identify and close care gaps across diverse patient populations. Multiple Accountable Care Organizations (ACOs) rely on timely and accurate care gap information to drive performance. The highest-performing ACOs are those that effectively use data from HIEs, eliminating the creation of redundant networks.

Funding and Sustainability

Current funding mechanisms are overly complex and vary significantly by state, creating a barrier that undermines the development of a cohesive national network. To ensure long-term viability, a simplified, transparent funding model is needed at the national level that establishes clear federal guidelines defining eligibility and expectations for HIE funding. This approach would reduce reliance on inconsistent state-level support while creating a more predictable financial foundation for network operations. There should also be the removal of state-level barriers and reliance on individual state involvement in funding decisions would help ensure a more equitable and scalable national infrastructure.